

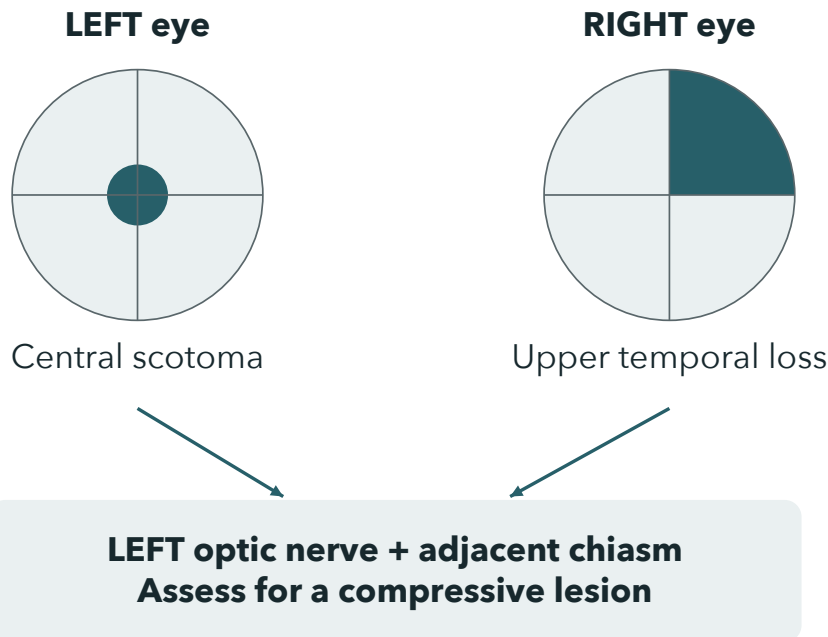
Visual fields and compressive localisation

CLINICAL TYPE B | Focused decisions for M3-M5

Read each field before joining the pattern

Asymmetry can point to the anterior junction

PATIENT VIEW: VISUAL FIELDS



Original field schematic viewed by the patient. Shading indicates lost visual space.

What two unequal fields can mean

Finding	Clinical inference
Left central loss + RAPD	Left optic-nerve dysfunction.
Right superior temporal loss	Adjacent crossing fibres are also affected.
The combined pattern	Consider the left optic-nerve/chiasm junction; target MRI to the sellar/anterior chiasmal region.

References AAO: Junctional scotoma | AAO: Compressive visual-field defects

Refine a homonymous defect

One hemisphere, two radiation divisions

RIGHT TEMPORAL radiation lesion



RIGHT PARIETAL radiation lesion



Original paired-field schematics. Right-sided lesions produce LEFT homonymous loss.

Radiation patterns

Finding	Clinical inference
Right temporal / Meyer loop	Left superior quadrantanopia.
Right parietal radiations	Left inferior quadrantanopia.
Broader right retrochiasmal lesion	May produce left homonymous hemianopia; field pattern alone cannot specify the exact lesion.

Use the whole examination

Vertical-meridian respect supports chiasmal/retrochiasmal localisation. Confirm field reliability and consider acuity, colour, RAPD and associated neurological signs; image the suspected pathway.